

hovid-Lamivudine Tablet 100 mg

VILAMye-PC2 (FR)

DESCRIPTION

Oblong, orange film-coated tablet, bevel-edged, shallow convex, "HOVID" embossed on one face and scored on the other face.

COMPOSITION

Each tablet contains: Lamivudine 100 mg

PHARMACODYNAMICS

Lamivudine belongs to a group of antiviral agents called nucleoside analogue reverse transcriptase inhibitors (NRTIs). Lamivudine is converted intracellularly to the triphosphate derivative which is the active form of the parent compound. This triphosphate inhibits the DNA synthesis of retroviruses, including HIV, through competitive inhibition of reverse transcriptase and incorporation into viral DNA. Lamivudine is also active against hepatitis B virus.

PHARMACOKINETICS

Lamivudine is rapidly absorbed after oral doses and peak plasma concentrations occur in about 1 hour. Absorption is delayed, but not reduced, by ingestion with food. Bioavailability is between 80 and 87%. Binding to plasma protein is reported to be up to 36%. Lamivudine crosses the blood-brain barrier with a ratio of CSF to serum concentrations of about 0.12. It crosses the placenta and is distributed into breast milk.

Lamivudine is metabolized intracellularly to the active antiviral triphosphate. Hepatic metabolism is low and it is cleared mainly unchanged by active renal excretion. An elimination half-life of 5 to 7 hours has been reported after a single dose.

INDICATION

Treatment of patients ≥ 16 years of age with chronic hepatitis B and evidence of hepatitis B virus (HBV) replication.

CONTRAINDICATIONS

Lamivudine is contraindicated in patients with hypersensitivity to the active substance or to any of the excipients.

WARNINGS AND PRECAUTIONS

- Lamivudine therapy should be stopped in patients who develop abdominal pain, nausea, or vomiting or with abnormal biochemical test results until pancreatitis has been excluded.
- Treatment with lamivudine may be associated with lactic acidosis and should be stopped if there is a rapid increase in aminotransferase concentrations, progressive hepatomegaly, or metabolic or lactic acidosis of unknown aetiology.
- Lamivudine should be used with caution in patients with hepatomegaly or other risk factors for hepatic disease.
- Patients co-infected with HIV and chronic hepatitis B or C and treated with combination antiretroviral therapy are at increased risk for severe and potentially fatal hepatic adverse events.
- In patients with chronic hepatitis B, there is a risk of rebound hepatitis when lamivudine is stopped, and liver function should be monitored in such patients.
- The possibility of HIV infection should be excluded before beginning lamivudine therapy for hepatitis B, since the lower doses used to treat the latter may permit the development of lamivudine-resistant strains of HIV.
- Effect on Driving and Use Machines:** No studies on the effects on the ability to drive and use machines have been performed.

PREGNANCY AND LACTATION

- Pregnancy:** There are no adequate and well-controlled studies of lamivudine tablets in pregnant women. Animal reproduction studies in rats and rabbits revealed no evidence of teratogenicity. Increased early embryolethality occurred in rabbits at exposure levels similar to those in humans. Lamivudine should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus.
- Lactation:** Lamivudine passes into breast milk. Because of the potential for serious adverse reactions in nursing infants, a decision should be made to discontinue hovid-Lamivudine Tablet 100mg taking into consideration the importance of continued hepatitis B therapy to the mother and the known benefits of breastfeeding.

DRUG INTERACTIONS

Renal excretion of lamivudine may be inhibited by other drugs mainly eliminated by active renal secretion, for example trimethoprim. Usual prophylactic doses of trimethoprim are unlikely to necessitate reductions in lamivudine dosage unless the patient has renal impairment, but the co-administration of lamivudine with the high doses of trimethoprim (as co-trimoxazole) used in pneumocystis pneumonia and toxoplasmosis should be avoided.

Although there is usually no clinically significant interaction with zidovudine, severe anemia has occasionally been reported in patients given lamivudine with zidovudine.

Lamivudine may antagonize the antiviral action of zalcitabine and the two drugs should not be used together.

In vitro lamivudine inhibits the intracellular phosphorylation of cladribine leading to a potential risk of cladribine loss of efficacy in case of combination in the clinical setting. Some clinical findings also support a possible interaction between lamivudine and cladribine. Therefore, the concomitant use of lamivudine with cladribine is not recommended.

Due to similarities, hovid-Lamivudine Tablet 100 mg should not be administered concomitantly with other cytidine analogues, such as emtricitabine. Moreover, hovid-Lamivudine Tablet 100 mg should not be taken with any other medicinal products containing lamivudine.

Once daily triple nucleoside regimens of lamivudine and tenofovir with either abacavir or didanosine are associated with a high level of treatment failure and of emergence of resistance, and should be avoided.

MAIN SIDE/ ADVERSE EFFECTS

Patients taking a lower dose of lamivudine for the treatment of chronic hepatitis B often have abdominal discomfort and pain, diarrhoea, fatigue, headache, nausea, malaise, respiratory tract infections and vomiting. The most frequently reported laboratory abnormalities are elevated creatine phosphokinase, increases in serum lipase, and raised liver enzymes, in particular alanine aminotransferase, and muscle disorders such as cramps, myalgia, and rhabdomyolysis.

OVERDOSE AND TREATMENT

Limited data are available on the consequences of ingestion of acute overdoses in humans. No fatalities occurred, and the patients recovered. No specific signs or symptoms have been identified following such overdose.

If overdose occurs the patient should be monitored and standard supportive treatment applied as required. Since lamivudine is dialyzable, continuous haemodialysis could be used in the treatment of overdose, although this has not been studied.

DOSAGE AND ADMINISTRATION

100mg once daily to be taken with or without food.

Patient compliance should be monitored while taking lamivudine. Discontinuation may be considered in immunocompetent patients when HBeAg and/or HBsAg seroconversion occur. Discontinuation may also be considered when loss of efficacy occurs, as indicated by recurrent signs of hepatitis.

After treatment discontinuation, patients should be periodically monitored for evidence of recurrent hepatitis.

Discontinuation of treatment is not recommended in patients with decompensated liver disease. There are limited data regarding the maintenance of long-term seroconversion after stopping treatment with lamivudine.

Renal impairment: Lamivudine serum concentrations (AUC) are increased in patients with moderate to severe renal impairment due to decreased renal clearance. The dosage should therefore be reduced in patients with creatinine clearance <50 ml/min.

Data available in patients undergoing intermittent haemodialysis (≤ 4 hrs dialysis 2-3 times weekly), indicate that following the initial dosage reduction of lamivudine to correct for the patient's creatinine clearance, no further dosage adjustments are required while undergoing dialysis.

Hepatic impairment: Data obtained in patients with hepatic impairment, including those with end-stage liver disease awaiting transplant, show that lamivudine pharmacokinetics are not significantly affected by hepatic dysfunction. Based on these data, no dose adjustment is necessary in patients with hepatic impairment unless accompanied by renal impairment.

Note: The information given here is limited. For further information, kindly consult your doctor or pharmacist.

Storage:
Store below 30°C.

Presentation/ Packing:
Blister pack of 1 x 10's, 3 x 10's, 10 x 10's

Product Registration Holder: HOVID Bhd.
121, Jalan Tunku Abdul Rahman
(Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Manufactured by: HOVID Bhd.
Lot 56442, 7 1/2 Miles, Jalan Ipoh/Chemor,
31200 Chemor, Perak, Malaysia.

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